

Clinical Study Summary Report (CSSR)

Document Status: Draft | Version: 1.0 | Date: 02-Apr-2026

1. Administrative & Study Identification

Full Study Title: Long-Term Real-World Outcomes Among Patients With Programmed Death-Ligand 1 (PD-L1) <1% Metastatic Non-Small Cell Lung Cancer (mNSCLC) Treated With First-Line (1L) Nivolumab + Ipilimumab + 2 Cycles Of Chemotherapy (NIC) **Short Title/ Acronym:** Long-term Outcomes Among Patients With PD-L1 <1% mNSCLC Treated With 1L Nivolumab + Ipilimumab + Chemotherapy **Posting ID:** 156584410482387063 **Protocol Number:** CA209-1496 **NCT Number:** NCT07024862 **Sponsor/Company Name:** Bristol Myers Squibb **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Investigational Product:** Biological: nivolumab (Trade Names: Opdualag, Opdivo; ATC Code: L01FF01) + ipilimumab **Phase of Development:** Not Applicable **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the long-term real-world outcomes among adults diagnosed with programmed death-ligand 1 (PD-L1) <1% metastatic non-small cell lung cancer (mNSCLC) treated with first-line (1L) nivolumab + ipilimumab + 2 cycles of chemotherapy (NIC) in the United States. **Secondary Objectives:** To evaluate additional real-world effectiveness and safety measures associated with the 1L NIC regimen in this specific patient population.

2.2 Background & Rationale To address the clinical necessity for long-term real-world evidence. While clinical trials establish efficacy, this observational study is conducted to evaluate the real-world effectiveness, survival, and long-term outcomes of the first-line nivolumab + ipilimumab combined with 2 cycles of platinum-doublet chemotherapy in mNSCLC patients who have negative PD-L1 expression (<1%) and wild-type genetics, providing critical data on how this regimen performs in standard clinical practice.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational **Control Type:** Single-arm (1 patient group/Cohort 1) **Blinding:** Open-label (N/A for observational retrospective design) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target **NS:** 100 estimated patients **Number of Sites:** United States (US-based) **Estimated**

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. At least 18 years old at confirmed diagnosis with metastatic non-small cell lung cancer (NSCLC) (stage IV), squamous or non-squamous histology. 2. Initiated first-line (1L) combination nivolumab, ipilimumab, and 2 cycles of platinum-doublet chemotherapy (NIC) regimen between 01 January 2018 and 2 years prior to study launch. 3. Tumor programmed death-ligand 1 (PD-L1) negative expression (i.e., PD-L1 <1%). 4. No EGFR, ALK, or ROS1 genetic mutation (i.e., wild type). 5. Minimum of 6 months of follow-up after initiation of 1L NIC.

4.2 Exclusion Criteria 1. Any prior systemic therapy for metastatic non-small cell lung cancer (NSCLC).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: First-line (1L) combination of nivolumab, ipilimumab, and 2 cycles of platinum-doublet chemotherapy administered based on real-world institutional standards. **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Retrospective/prospective observation based on standard real-world treatment duration.

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant medications and supportive care for mNSCLC allowed in standard clinical practice. **Prohibited:** Prior systemic therapy for metastatic NSCLC.

6. Study Timeline & Visit Schedule

(Note: As an observational real-world outcomes study, timeline reflects data abstraction rather than clinical visits)

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Index Date	Day 0 Initiation of 1L NIC regimen (Retrospective Data Abstraction) Observation
Minimum 6 Months Retrospective evaluation of real-world outcomes and survival End of Study Jun 2025 Final Data Extraction & primary completion			

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. Participant baseline socio-demographics 2. Participant baseline clinical characteristics 3. Participant treatment history **Measurement Method:** Retrospective/

prospective review and abstraction of electronic health records (EHR) or patient charts.

7.2 Secondary & Exploratory Endpoints Real-world Progression-Free Survival (rwPFS). Time to Next Treatment (TTNT). Real-world Overall Response Rate (rwORR).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Retrospective abstraction of documented adverse events from patient medical records. **Serious Adverse Events (SAEs):** Evaluated as recorded in standard real-world oncology care. **Data Monitoring Committee (DMC):** N/A (Observational study).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Real-World Evidence (RWE) Cohort (All enrolled eligible patients meeting selection criteria). **Sample Size Justification:** Target $N = 100$ estimated based on the available cohort of patients meeting the specific criteria in the US. **Handling of Missing Data:** Appropriate statistical methods for missing data in retrospective observational datasets (e.g., multiple imputation).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and applicable regulatory guidelines for observational research. **Monitoring Plan:** Source data verification of abstracted electronic health records (EHR). **Audit Trail:** Secure database with standard audit trails for all data points abstracted.

11. Study Identifiers & Governance

Primary ID: CA209-1496 **Posting ID:** 156584410482387063 **Registry Number:** NCT07024862 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** Long-term Outcomes Among Patients With Programmed Death-ligand 1 <1% Metastatic Non-small Cell Lung Cancer Treated With First-line Nivolumab + Ipilimumab + 2 Cycles of Chemotherapy **Official Regulatory Title:** Long-Term Real-World Outcomes Among Patients With Programmed Death-Ligand 1 (PD-L1) <1% Metastatic Non-Small Cell Lung Cancer (mNSCLC) Treated With First-Line (1L) Nivolumab + Ipilimumab + 2 Cycles Of Chemotherapy (NIC)

12. Clinical Framework (mNSCLC Specific)

Primary Condition: Metastatic Non-small Cell Lung Cancer (mNSCLC) (MeSH Code: D016938 | SNOMED ID: 457721000124104) **Diagnosis Threshold:** Stage IV, PD-L1 <1%, EGFR/ALK/ROS1 wild type **Medical Methodology:** First-Line combination nivolumab + ipilimumab + 2 cycles of platinum-doublet chemotherapy **Optimization Target:** Long-term real-world outcomes and survival

13. Study Procedures & Methodology

Management Pathway: Real-world standard clinical pathway for mNSCLC **Education Protocol (HCP):** N/A - Observational study **Education Protocol (Patient):** N/A - Observational study **Quality Audit Mechanism:** Electronic health record (EHR) data verification and abstraction review

14. Observation & Follow-Up Schedule

Total Duration: Minimum of 6 months of follow-up after initiation of 1L NIC **Onsite Visit Cadence:** N/A - Observational retrospective data collection **Remote Monitoring Frequency:** N/A **Checklist Review Interval:** N/A

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				